Clinical Study Summary Report (CSSR)
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1. Administrative & Study Identification

Full Study Title: A Study to Learn About the Study Medicine - Hympavzi in Congenital Hemophilia Patients Without Inhibitors in Japan. **Official Regulatory Title:** HYMPAVZI S.C. INJECTION 150 mg Pen SPECIAL INVESTIGATION **Short Title/Acronym:** Hympavzi Special Investigation
Primary ID: Not Available **Protocol Number:** [Not Available / Post-Marketing Registry]
Registry Number (NCT Number): NCT07161687 **Sponsor/Lead Organization:** Pfizer **Investigational Product:** Hympavzi (marstacimab) S.C. INJECTION 150 mg Pen **Phase of Development:** Phase IV / Not Applicable (Observational Special Investigation)
Medical Monitor: Pfizer Pharmacovigilance / Medical Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the long-term safety of Hympavzi under actual usage conditions in patients with congenital hemophilia who do not have inhibitors. **Secondary Objectives:** To evaluate the occurrence of treatment-related adverse drug reactions (ADRs) and serious adverse drug reactions (SADRs) over a 3-year observation period, and to monitor overall clinical effectiveness with an optimization target of a reduction in treated Annualized Bleeding Rate (ABR).

2.2 Background & Rationale Hemophilia A and B are life-threatening congenital bleeding disorders traditionally managed with frequent intravenous factor replacement therapies. Hympavzi (marstacimab) is a monoclonal antibody targeting the tissue factor pathway inhibitor (TFPI) to rebalance hemostasis, offering routine non-factor prophylaxis via a straightforward, once-weekly subcutaneous injection. This special investigation is conducted to gather real-world safety, long-term tolerability, and effectiveness data under actual clinical practice settings in Japan for patients 12 years and older.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Special Investigation / Post-Marketing Surveillance) **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A **Management Pathway:** Subcutaneous prophylaxis administration

3.2 Planned Enrollment & Duration Target N: Real-world patient registry (target sample size based on actual use)

Number of Sites: Multiple clinical sites across Japan **Estimated Study Start:** 2025 **Estimated Primary Completion:** Up to 156 weeks (3 years) of evaluation per participant

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Primary Condition: Diagnosis of congenital hemophilia A or B. 2. Diagnosis Threshold: Absence of FVIII or FIX inhibitors. 3. Age ≥ 12 years. 4. Body weight ≥ 35 kg.

4.2 Exclusion Criteria 1. Patients with a history of or current FVIII or FIX inhibitors. 2. Age < 12 years. 3. Body weight < 35 kg.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Administered subcutaneously with an initial loading dose of 300 mg, followed by 150 mg once weekly. If a patient weighs ≥ 50 kg and shows an inadequate response, the dose may be escalated to 300 mg once weekly. **Route of Administration:** Subcutaneous (S.C.) injection via Pen. **Medical Methodology:** Routine Non-Factor Prophylaxis. **Duration of Treatment:** Up to 156 weeks (3 years) of observation.

5.2 Concomitant & Prohibited Therapy Permitted: Standard on-demand treatments for breakthrough bleeding episodes as prescribed by the treating physician. **Prohibited:** Routine prophylaxis with concurrent bypassing agents or other monoclonal antibodies conflicting with anti-TFPI therapy.

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6. Study Timeline & Visit Schedule

Total Duration: 156 Follow-up weeks (3 years) **Onsite Visit Cadence:** Routine clinical follow-ups per standard of care **Checklist Review Interval:** Periodic physician review of ADRs and bleeding logs

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent, Medical History, Confirmation of Hemophilia Status & Inhibitor Absence
Baseline	Day 0	First Dose Administration (300 mg); Patient Education Protocol: Pen injector training and self-administration instructions; HCP Education Protocol: Instructions on proper S.C. injection and pharmacovigilance safety reporting
Interim	Standard of Care	Routine clinical visits: Safety assessment, ABR tracking, ADR/SADR evaluation
End of Study	Week 156	Final Safety Check, Event Documentation (or at the point of premature discontinuation)

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Occurrence, incidence, and severity of Adverse Drug Reactions (ADRs) and Serious Adverse Drug Reactions (SADRs) from the first dose up to 156 weeks (3 years). **Measurement Method:** Clinical observation and physician assessment of relatedness to Hympavzi under actual use in Japan.

7.2 Secondary & Exploratory Endpoints Treated Annualized Bleeding Rate (ABR) under actual usage. Discontinuation rates and analysis of reasons for early treatment withdrawal.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Physician tracking of any untoward medical occurrence attributed to Hympavzi. **Serious Adverse Events (SAEs):** Reported immediately for outcomes including death, life-threatening events, initial or prolonged inpatient hospitalization, persistent/significant disability, or congenital anomaly. **Remote Monitoring Frequency:** Continuous safety and bleeding event reporting. **Data Monitoring Committee (DMC):** Pharmacovigilance safety monitoring governed by the sponsor (Pfizer) in compliance with Japanese regulatory standards.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all enrolled patients who received at least one dose of Hympavzi). **Sample Size Justification:** Registry based on actual drug usage; formal power calculations are not applicable for this open-label surveillance. **Handling of Missing Data:** For patients discontinuing treatment early, safety and efficacy data will be collected and analyzed up to the point of discontinuation.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Vigilance Practice (GVP) and Good Post-Marketing Study Practice (GPSP) in Japan. **Monitoring Plan:** Continuous post-marketing safety data collection throughout the 3-year evaluation period. **Quality Audit Mechanism:** Long-term adverse drug reaction (ADR) tracking and real-world safety monitoring. **Audit Trail:** Standard tracking of pharmacovigilance reports and electronic data capture for SADRs.

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11. Study Identifiers & Governance

Primary ID: Not Available **Registry Number:** NCT07161687 **Sponsor/ Lead Organization:** Pfizer **Study Title:** A Study to Learn About the Study Medicine -Hympavzi in Congenital Hemophilia Patients Without Inhibitors in Japan **Official Regulatory Title:** HYMPAVZI S.C. INJECTION 150 mg Pen SPECIAL INVESTIGATION

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12. Clinical Framework (Hemophilia Specific)

Primary Condition: Congenital Hemophilia A or B **Diagnosis Threshold:** Absence of FVIII or FIX inhibitors **Medical Methodology:** Routine Non-Factor Prophylaxis **Optimization Target:** Reduction in treated Annualized Bleeding Rate (ABR)

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13. Study Procedures & Methodology

Management Pathway: Subcutaneous prophylaxis administration **Education Protocol (HCP):** Instructions on proper S.C. injection and pharmacovigilance safety reporting **Education Protocol (Patient):** Pen injector training and self-administration instructions **Quality Audit Mechanism:** Long-term adverse drug reaction (ADR) tracking and real-world safety monitoring

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14. Observation & Follow-Up Schedule

Total Duration: 156 Follow-up weeks (3 years) **Onsite Visit Cadence:** Routine clinical follow-ups per standard of care **Remote Monitoring Frequency:** Continuous safety and bleeding event reporting **Checklist Review Interval:** Periodic physician review of ADRs and bleeding logs

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	**[DD-MMM-YYYY]**
Clinical Operations Lead	**[Name]**	___	***[DD-MMM-YYYY]***
Lead Statistician	***[Name]***	_____	[DD-MMM-YYYY]